
Comprehensive Operational Manual

Customer Care & Patient Journey

Patient Acquisition • Retention • Schedule Optimization • Cash Flow Preservation

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Executive Operating Snapshot

Management Objective

Map the complete Customer Care operating model from first patient contact through scheduling, insurance control, attendance management, discharge, and billing readiness. Every rule in this manual protects one of four outcomes:

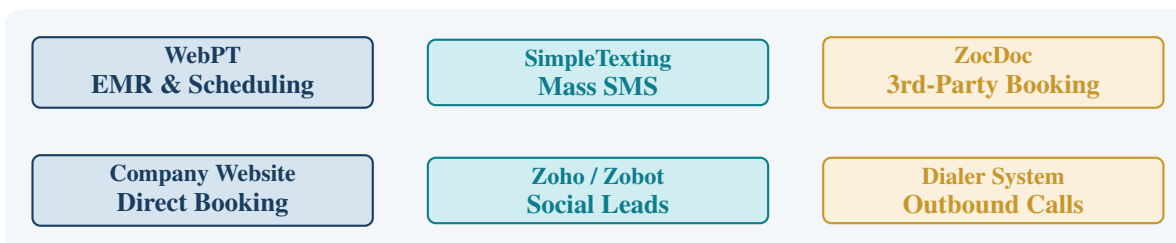
Acquisition	Retention	Capacity	Cash Flow
New lead capture	Future visits booked	Green Board protected	Denials prevented

Operating Principle

Customer Care is not only a communication function. It is a revenue-control function. An incorrect case selection, expired authorization, missed hold status, or incomplete check-out can convert a treated visit into an unbillable visit.

1 Software & Tool Ecosystem

The department operates through a tightly integrated suite of platforms. Each tool serves a specific role in the patient lifecycle pipeline.

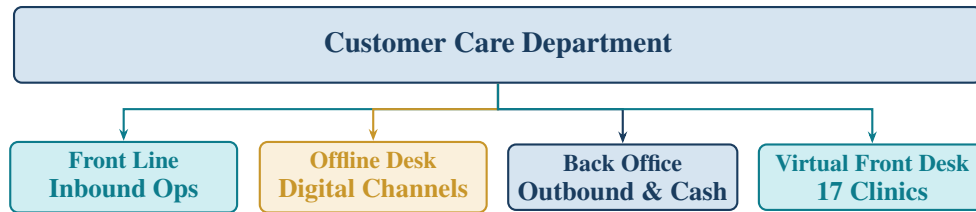


Tool Reference Table

Tool	Function	Primary User
WebPT	EMR: chart creation, Green Board scheduling, clinical documentation	All teams
SimpleTexting	Mass SMS confirmations (C / R / X campaigns)	Back Office
ZocDoc	Third-party self-booking portal (similar to Vezeeta)	Patients / Offline Desk
Website	Direct booking with demographics & insurance upload	Patients / Offline Desk
Zoho (Zobot)	Lead management from social media (Instagram, Twitter, LinkedIn)	Offline Desk
Dialer	Outbound call management & disposition tracking	Back Office / Front Line

2 Departmental Structure & Pillars

The Customer Care department is divided into **four operational lines of business**.



2.1 Front Line — Inbound Operations

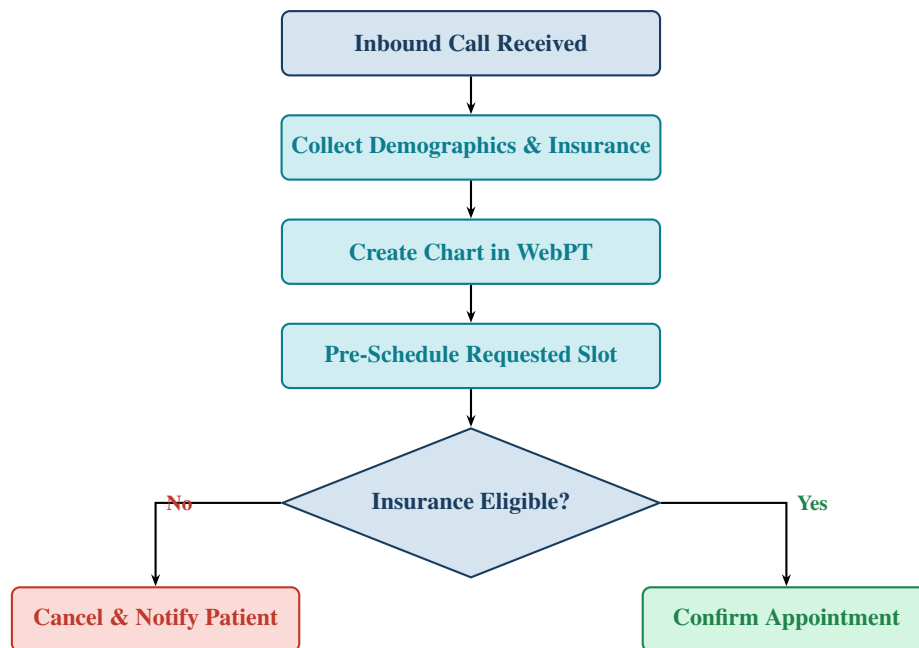
The Front Line team receives all inbound calls and is responsible for new patient onboarding following a strict sequential order.

Front Line SOP — Sequential Steps

- 1. Patient Intake:** Collect full demographic, contact, referral, and insurance information.
- 2. Chart Creation:** Generate a new chart in WebPT *before* any scheduling action.
- 3. Case Accuracy Check:** If the patient is returning, confirm whether the visit belongs to an existing active case or a new case.
- 4. Pre-Scheduling:** Secure the requested time slot *before* eligibility is finalized to protect the lead.
- 5. Eligibility Check:** Coordinate with the Benefits Team to verify active coverage, authorization rules, and visit limits.
- 6. Patient Notification:** Confirm eligible appointments; cancel and notify ineligible patients before treatment occurs.

Front Line Billing Control Point

The highest-risk intake mistake is scheduling a returning patient under the wrong WebPT case. If the patient is being treated for a new body part, the appointment must not be attached to a closed or unrelated case.



2.2 Offline Desk — Digital Channel Operations

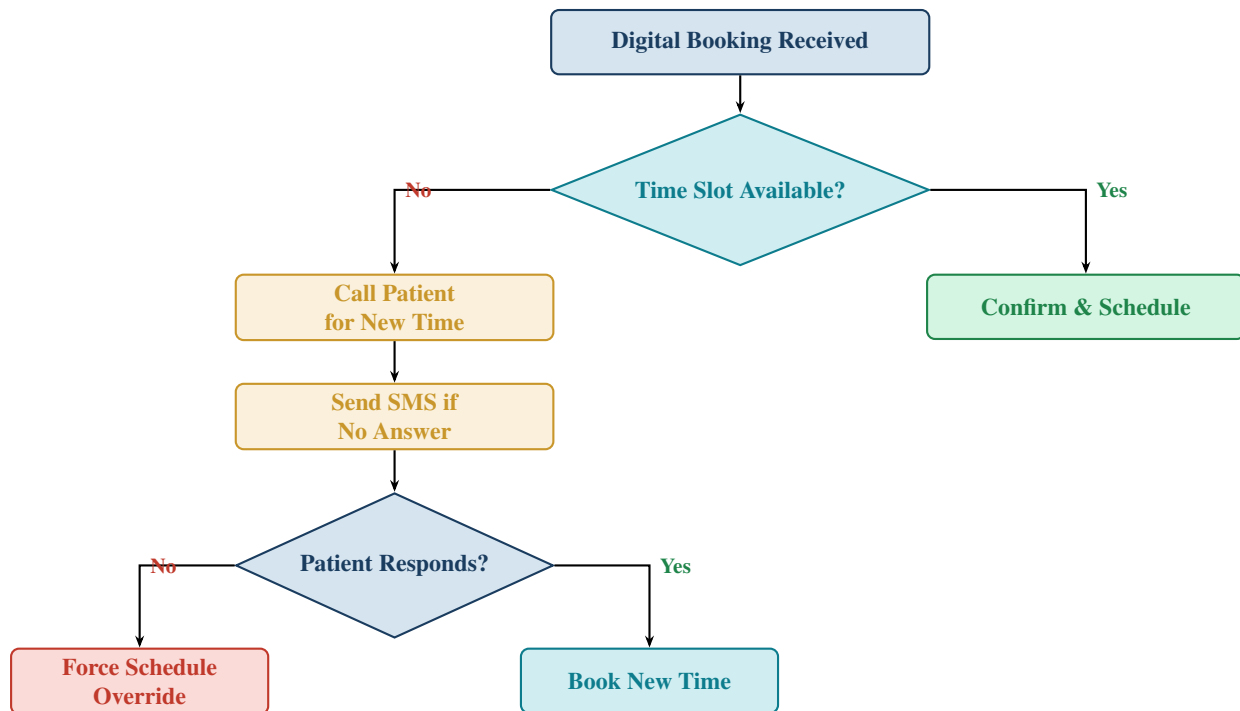
This team manages all asynchronous digital bookings from ZocDoc, the website, and social media.

Offline Desk Key Rules

- **Source Coverage:** ZocDoc, company website, and Zoho/Zobot social leads are all treated as active booking queues.
- **Queue Management:** Every requested time is cross-referenced against WebPT availability and Green Board capacity.
- **Conflict Resolution:** If time unavailable — call first, then SMS if no answer.
- **Force Scheduling Exception:** If patient remains unresponsive, override only as a lead-preservation exception and flag for follow-up.
- **Hard Deadline:** Queue *must* be closed by **3:00 PM** daily.

Offline Desk 3:00 PM Closure Rule

The 3:00 PM deadline is an operational control, not an administrative preference. If the digital queue remains open after 3:00 PM, next-day schedules may show inaccurate capacity, unconfirmed arrivals, or unresolved conflicts.



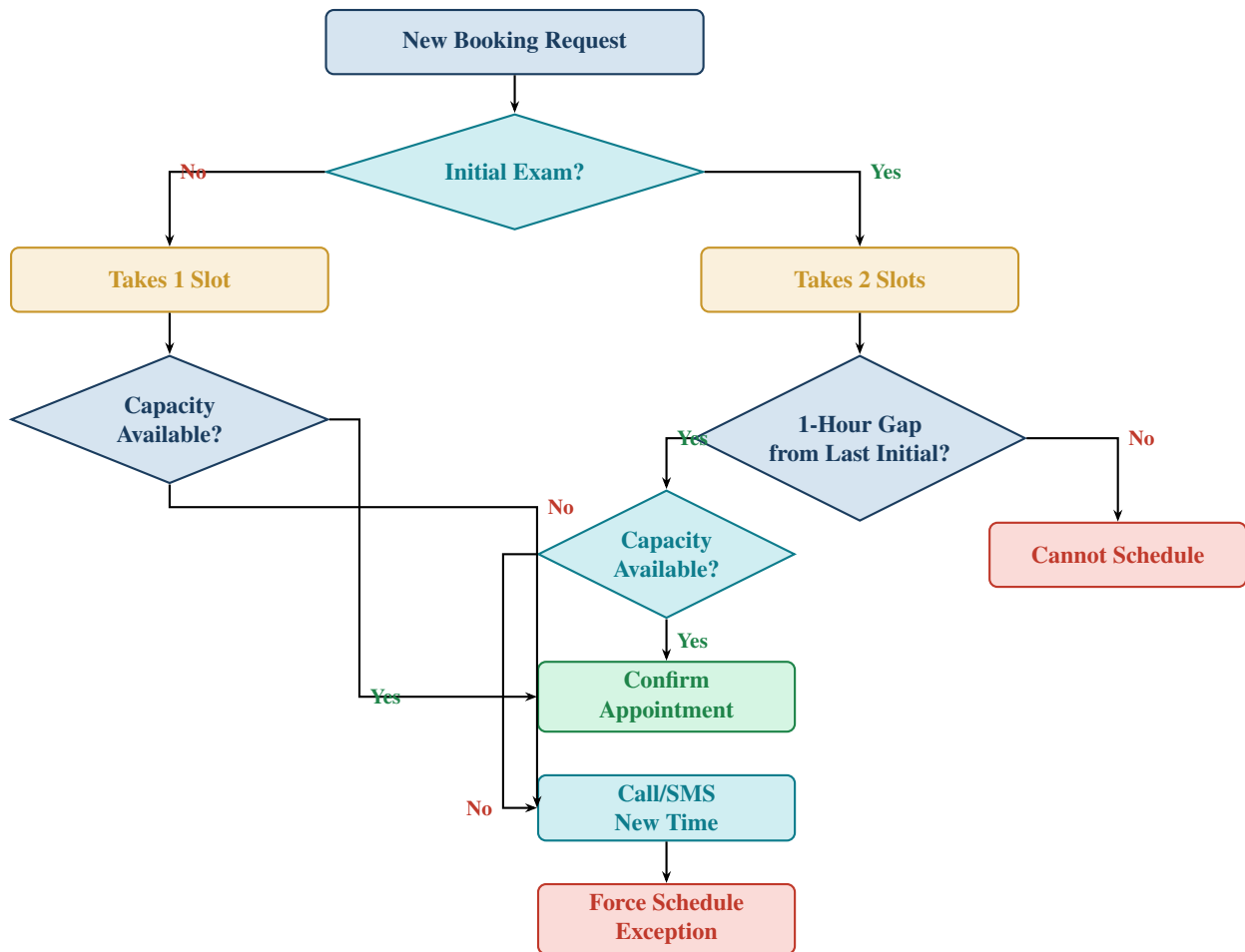
2.3 Green Board Scheduling Rules

Scheduling is governed by the **Green Board** — strict clinical capacity guidelines dictating how many patients a PT can see per time block.

Green Board Capacity Rules

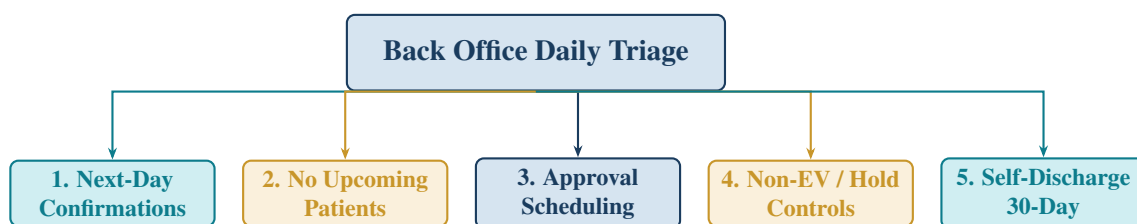
Visit Type	Rule
Follow-up	1 slot (30-minute unit)
Initial Exam	2 slots (equals 2 follow-ups)
Re-examination	Special type — highlighted red in WebPT

Restriction	Rule
Back-to-back Initials	PROHIBITED — must have 1-hour gap
Simultaneous Initials	Allowed only if total capacity permits
Capacity Breach	3 follow-ups + 1 Initial = 5 slots → VIOLATION



2.4 Back Office — Outbound & Cash Flow Operations

The Back Office is the cash-flow engine, executing five critical daily workflows that protect attendance, authorization compliance, and future booking continuity.



2.4.1 Workflow 1: Next-Day Confirmations

Confirmation Cycle — Step by Step

1. **Mass SMS Blast:** Send campaign via SimpleTexting to all next-day patients using the C/R/X instruction format.
2. **2-Hour Hold:** Wait two hours before filtering and actioning responses.
3. **Action Responses:**
 - **C / Confirm:** Mark as confirmed on schedule.
 - **R (Reschedule):** Cancel current; book new appointment immediately.

- **X (Cancel):** Document reason. If patient has future appointments → keep. If no future appointments → attempt to schedule; document failure reason if unsuccessful.

4. Outbound Calls: Call all non-respondents directly.

5. Audit Trail: Every opened SMS thread must be actioned, logged, or assigned for follow-up before leaving the queue.

SimpleTexting Search Rule

Do not search only for the single letter “C”. Patients may reply with “**Confirm**”, “**confirmed**”, or other natural-language responses. The agent must review opened threads and not rely only on exact-letter filtering.

Key Statistical Reality

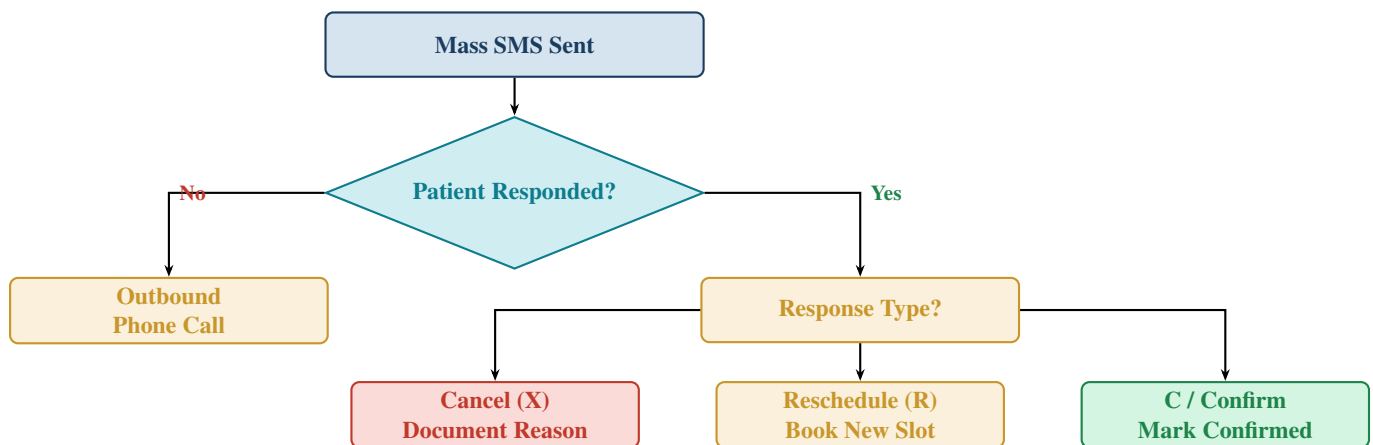
62%

of non-respondents
still show up

20%

of confirmed patients
still No-Show

Confirmation ≠ Attendance. Non-response ≠ Cancellation.

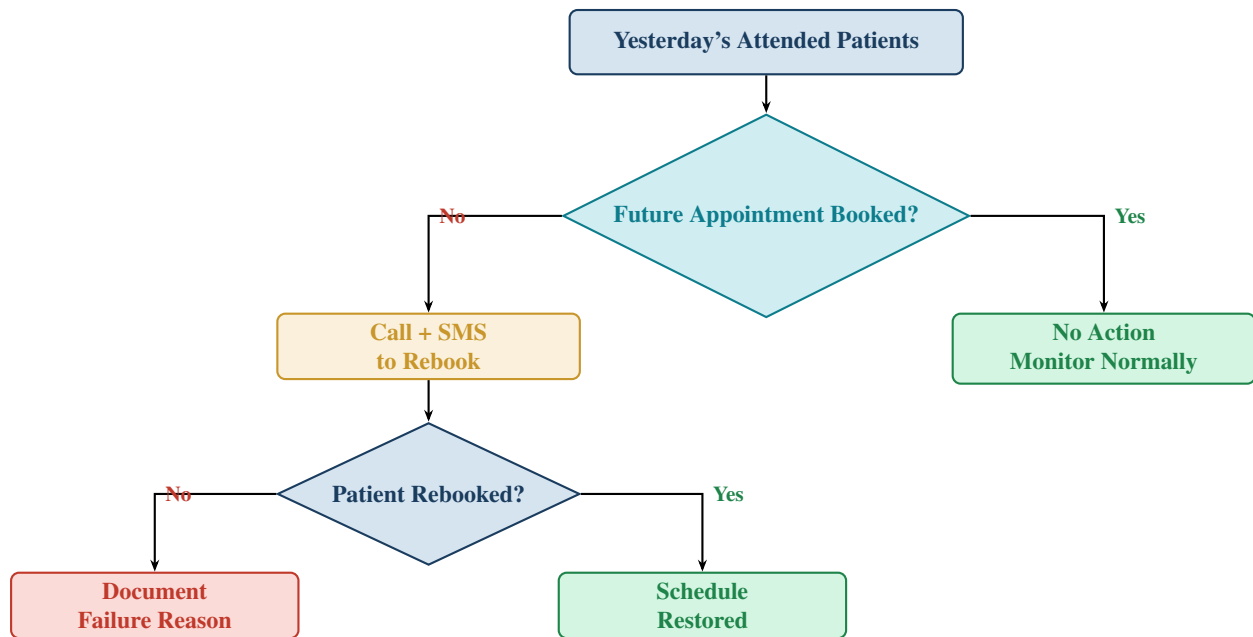


2.4.2 Workflow 2: No Upcoming Appointments

Agents pull a daily list of patients who attended a session yesterday but left without booking their next appointment. They proactively call *and* SMS these patients to re-engage them before the gap widens.

No Upcoming Control Rule

This workflow protects retention and visit continuity. A patient who leaves the clinic without a future appointment is a revenue-risk patient, even if they attended yesterday.



2.4.3 Workflow 3: Insurance Approval Scheduling

Approval updates represent immediate scheduling opportunities. Agents review yesterday's approvals daily; on Mondays, they review the full weekend backlog.

Approval Scheduling SOP

1. Pull all new authorization approvals from the Authorization Team update.
2. Confirm visit count, authorization window, expiration date, and case association.
3. Call the patient immediately to schedule approved visits.
4. If the patient does not answer, send SMS and document the outreach attempt.
5. Avoid booking beyond the approved authorization window.

2.4.4 Workflow 4: Non-EV and Hold Patient Controls

Insurance Status Management Rules

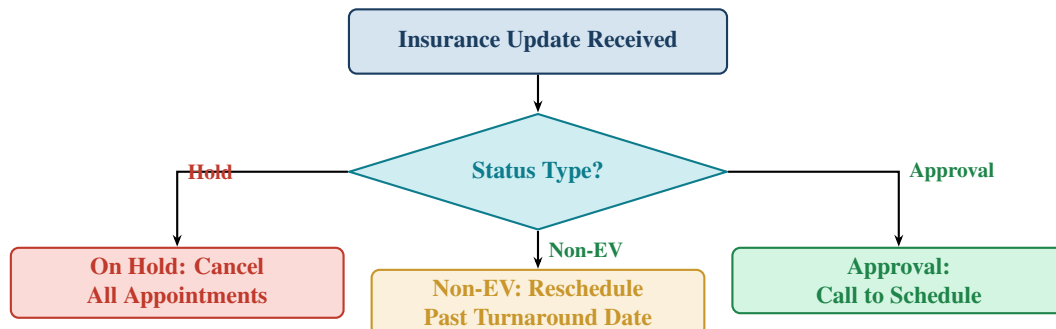
Status	Description	Action
Approval	New authorized visits granted (Mon: review full weekend)	Call immediately to schedule
Non-EV	Visits exhausted; awaiting turnaround (e.g., 4-day cycle)	Reschedule PAST turnaround date
On Hold	Missing referral, COB issue, or insurance change	Cancel ALL upcoming appts

Critical Rule — Hold Patients

Agents **MUST cancel all upcoming appointments** for On Hold patients. Failure = clinic treats patient for free = unbillable visit = direct revenue loss.

Non-EV Rule

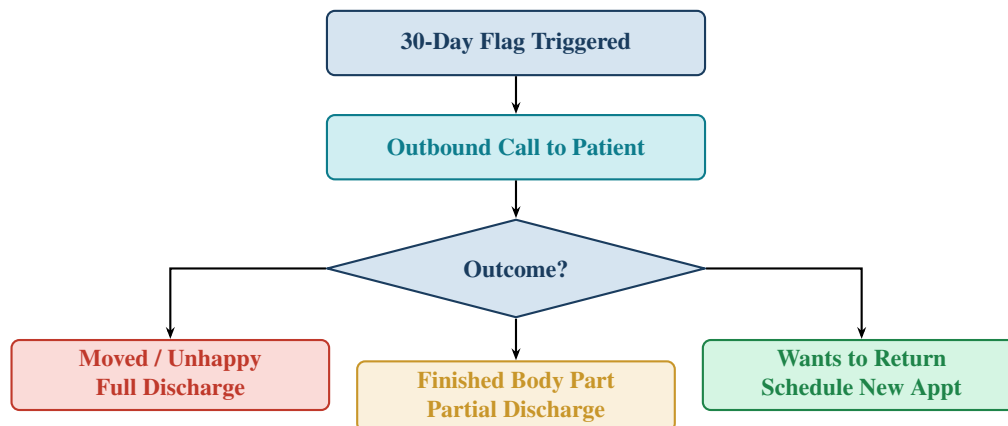
Agents **MUST NOT cancel** Non-EV patients. Reschedule to a date *after* the expected insurance turnaround window.

**2.4.5 Workflow 5: Self-Discharge — Lost Patients (30-Day)**

Patients not attending for **30 days** are flagged as *Self-Discharged*. Agents call to uncover the reason.

Self-Discharge Reasons & Actions

Reason	Required Action
Medical issue / recovering	Partial or full discharge
Relocated	Full discharge — close chart
Unhappy with clinic / PT	Full discharge — document feedback
Finished specific body part	Partial discharge — close case only; may return as New Case
Fully recovered	Full discharge
Wants to return	Schedule new appointment immediately

**2.5 Virtual Front Desk (VFD)**

The VFD operates as a remote receptionist for **17 clinics** that lack physical front desk staff.

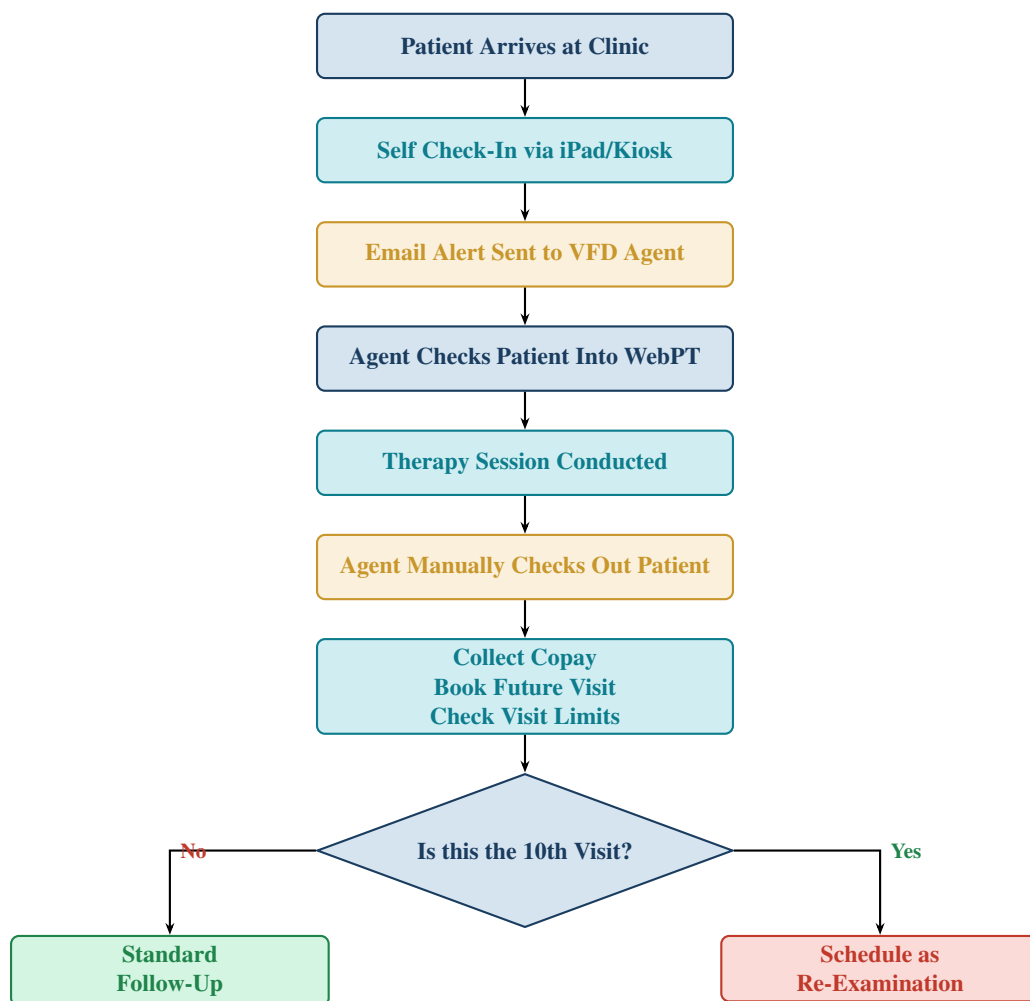
VFD Operational Responsibilities

- **Check-In:** Patient uses iPad/Kiosk (Name + DOB) → email triggers VFD agent → manual check-in in WebPT.
- **Check-Out (Intentionally Manual):** Self check-out is disabled by design. Agents enforce:

1. **Copay Collection:** Activate POS machine.
 2. **Future Booking:** Verify or book next appointment.
 3. **Visit Limit Warning:** Alert patient on last authorised visit; place on brief hold.
- **Re-Examination:** Every **10th visit** must be scheduled as a *Re-examination* (shown **red** in WebPT), not a standard follow-up.

VFD Legal and Billing Control

If a patient is checked in but not checked out, WebPT can show the patient as still present in the clinic. This delays billing, disrupts operational reporting, and creates legal exposure regarding the patient's recorded whereabouts.

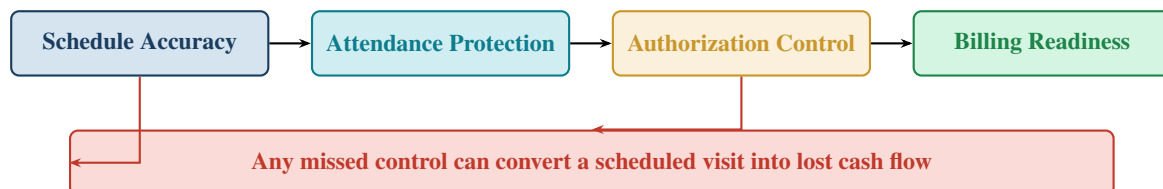


3 Daily Operating Control Dashboard

This dashboard converts the workflows into management checkpoints for daily review, staff supervision, and cash-flow forecasting.

Daily Control Points

Control	Owner	Required Evidence	Risk if Missed
Digital Queue Closure	Offline Desk	Queue cleared by 3:00 PM; unresolved conflicts documented	Schedule distortion
Next-Day Confirmations	Back Office	C/R/X responses processed; non-respondents called	No-Show leakage
No Upcoming Review	Back Office	Yesterday's attended-without-future list called and SMSed	Retention loss
Authorization Updates	Back Office	New approvals scheduled; Non-EV rescheduled; Hold canceled	Unbillable visits
VFD Check-Out	VFD	Copay, future visit, and visit-limit check completed	Billing/legal exposure
Case Selection Accuracy	Front Line / VFD	Correct active or new case selected in WebPT	Instant denial



4 Systemic Errors & Cash Flow Leakage Points

Each of the following errors leads directly to denied claims, unbillable visits, or lost revenue.

Error #1 — Multiple Cases Scheduling (Critical Billing Failure)

What happens: Patient returns for a new issue (e.g., Right Shoulder) but agent schedules under the old closed case (e.g., Left Knee) via the WebPT dropdown.

System: WebPT allows the booking without warning.

Consequence: Insurance claim **instantly denied** upon submission.

Error #2 — Expired Authorization Scheduling

What happens: Agent books a date past the insurance authorization expiry.

Example: Booking May 27th when authorization expired May 20th.

Consequence: Visit is **completely unbillable** — direct revenue loss.

Error #3 — Dummy Authorization Misinterpretation

What happens: Some insurers (e.g., Oxford Commercial) don't require formal auth. Billing enters "0" as placeholder. Agent misreads "0" as zero visits and cancels the patient.

Consequence: Revenue lost and patient relationship damaged unnecessarily.

Error #4 — SMS Leakage & Response Processing Failures

- Agent opens a message in SimpleTexting but fails to log or follow up.
- Searching for "C" misses patients who typed the full word "Confirm".

Consequence: No-Show rates increase; confirmed slots remain at financial risk.

Error #5 — Check-In / Check-Out Negligence (Legal Risk)

What happens: VFD agent checks in but forgets to check out.

System appearance: Patient appears in clinic **overnight**.

Consequences:

- Billing delays and claim submission failures.
- **Severe legal liability** regarding unaccounted patient whereabouts.

Error #6 — Missing Medical Notes (Billing Halt)

What happens: PT forgets to close/sign daily note in WebPT.

Consequence: Billing cannot mark claim as "**Ready for Submission**" — full cash flow halt for that visit.

Error #7 — Inaccurate Eligibility Communication

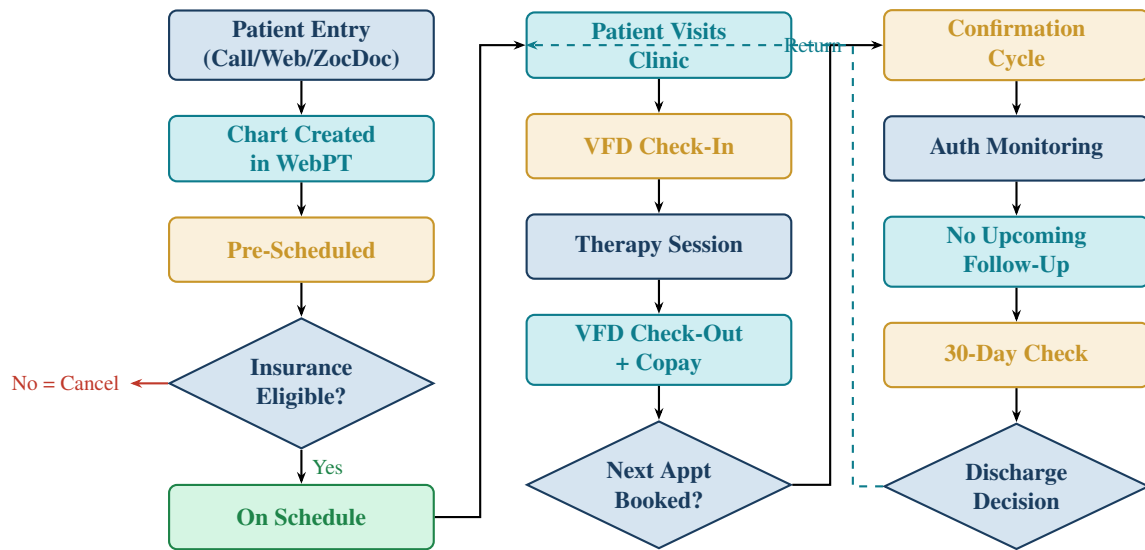
What happens: Patient deemed ineligible but agent fails to cancel upcoming appointment.

Consequence: Clinic treats patient for free — **fully unbillable visit**.

Error Risk Summary Matrix

Error	Severity	Preventive Control	Primary Impact
Multiple Cases Error	Critical	Verify active/new case before booking	Instant claim denial
Expired Authorization	Critical	Check authorization window and expiry date	Unbillable visit
Dummy Auth Misread	High	Confirm payer rule before cancellation	Lost revenue + patient churn
SMS Leakage	High	Review opened threads and natural replies	Increased No-Show rate
Check-In/Out Negligence	Critical	Manual check-out completion audit	Legal liability + billing delay
Missing Medical Notes	High	PT note closure escalation	Cash flow halt
Eligibility Communication Gap	Critical	Cancel ineligible appointments immediately	Unbillable visit

5 Full Patient Lifecycle — End-to-End Overview



Core Objective

Patient acquisition, retention, schedule optimisation, and cash flow preservation through rigorous compliance with billing and authorisation protocols.

Every scheduling decision, confirmation action, and discharge classification has a direct financial consequence.